

equicenta® CTM vs. ProVet® APC

Two very different products often considered for the same joint cases. This sheet lays out how they compare on mechanism, indications, evidence base, and the practical decisions vets and clients care about.

KEY REFERENCE

Bertone AL, et al. Cryopreserved umbilical allograft (equicenta® CTM) improves clinical outcomes compared with clinician-selected standard care in performance horses. *Frontiers in Veterinary Science* (2026). DOI: [10.3389/fvets.2026.1833933](https://doi.org/10.3389/fvets.2026.1833933)

	equicenta® CTM Cryopreserved equine umbilical cord tissue matrix (biologic allograft)	ProVet® APC Autologous platelet concentrate — leukocyte-rich PRP (Dechra, stall-side device)
Category	equicenta® CTM	ProVet® APC
Product class What it is	Allogeneic biologic. Cryopreserved equine umbilical cord tissue matrix with native ECM proteins, growth factors, and extracellular vesicles. Off-the-shelf.	Autologous cellular therapy. Leukocyte-rich platelet-rich plasma (L-PRP) prepared stall-side from a small blood draw using the ProVet centrifuge system. (Dechra IFU; Am J Vet Res 2024)
Mechanism of action Primary MOA	Biologic: supports tissue repair and modulates inflammation via a multi-modal payload of native ECM proteins, growth factors, and small “messenger” particles released as the matrix biodegrades.	Platelet + leukocyte concentrate: delivers elevated IL-1Ra (with a high IL-1Ra-to-IL-1β ratio) and elevated TGF-β1, plus platelet-derived growth factors, to modulate joint inflammation. (Am J Vet Res 2024 characterization study)
Site of action Where it works	Acts across joint tissues (synovium, cartilage matrix, peri-articular soft tissue) via biologic signaling; also used peri-tendinously and peri-ligamentously.	Cleared for stall-side preparation of an autologous platelet concentrate. Clinical use includes intra-articular joint injection and soft-tissue applications at the treating veterinarian’s discretion. (Dechra IFU)
Labeled / indicated uses Where each is used	Studied in a mixed real-world population of joint, tendon, and ligament conditions in performance horses across 8 U.S. equine specialty practices.	Cleared as a medical device to safely and rapidly prepare autologous platelet concentrate from a small blood sample at the patient’s point of care. The device itself is indication-agnostic; specific clinical applications are at vet discretion. (Dechra ProVet APC IFU)
Tissue scope in evidence Joints vs. soft tissue	Joints, tendons, and ligaments reported in the same pragmatic controlled trial (195 conditions).	Characterization published (Am J Vet Res 2024). Peer-reviewed equine efficacy trials for ProVet APC specifically are limited relative to the broader PRP literature. Practice-based use is widespread across joint and soft-tissue cases.
Autologous vs. allogeneic Consistency of starting material	Allogeneic, standardized. Same starting material every dose — removes patient-to-patient variability.	Autologous. Composition depends on the donor horse’s platelet and leukocyte counts, hydration, and comorbidities at time of collection. As a leukocyte-rich PRP, WBC content varies with the horse.
Peer-reviewed evidence Anchor publications	Characterization & 42-day biocompatibility (BMC Medicine 2025); EV/exosome characterization (ACVSMR 2026 poster); Bertone et al. 2026 pragmatic controlled multicenter clinical trial (Frontiers Vet Sci).	Am J Vet Res 2024 characterization: ProVet APC is a leukocyte-rich PRP with elevated IL-1Ra:IL-1β ratio and TGF-β1 (n=6 horses). The manufacturer’s IFU notes safety and effectiveness “have not been established” for bone healing, hemostasis, or other clinical use. (PubMed 41791222; Dechra IFU)
Study design context How the evidence was generated	Pragmatic multicenter controlled trial: 138 horses / 195 conditions / 8 practices; natural-disease enrollment; open-label; active-comparator vs. clinician-selected standard of care; blinded statisticians & imaging readers.	Anchor equine ProVet APC publication is a characterization study (quantifying cytokine and growth-factor content in the output), not a clinical efficacy trial. Additional efficacy evidence draws on the broader L-PRP literature rather than product-specific RCTs.
Clinical outcome signals How improvement is characterized	Faster and greater improvement in lameness, pain, and swelling vs. SOC; arm-level return-to-performance 52.9% vs. 29.3% at 26 wk (OR 2.72; p<0.006). Owner-reported body condition, coat, overall health, and QoL also favored eCTM (p<0.03).	Rationale is mechanistic: elevated IL-1Ra:IL-1β ratio and TGF-β1 in the product output. Clinical outcome data specific to ProVet APC in horses is limited in the peer-reviewed literature relative to older L-PRP work.
Onset & durability Timing profile	Separation from SOC by week 2 (pain, swelling) and week 4 (lameness); sustained through 12 wk (p<0.001); performance signal durable at 26 weeks.	Onset and durability follow general PRP experience — onset typically 1–4 weeks; durability variable. No published 26-week active-comparator return-to-performance data for ProVet APC specifically in the peer-reviewed equine record.

Category	equicenta® CTM	ProVet® APC
Dosing & workflow Rep-relevant practical facts	Off-the-shelf, ready-to-use, single-dose in most cases. No blood draw, no in-clinic processing time.	Stall-side autologous processing: small blood draw + <3-minute centrifugation on the portable ProVet centrifuge (~4.4 lbs). Collection, processing, and reinjection all done in one visit. Practice must own the ProVet centrifuge and the disposable per-treatment kit. (Dechra Canada product page; Dechra IFU)
Safety profile Comparative safety	Peer-reviewed 42-day biocompatibility (BMC Medicine 2025). In Bertone 2026, eCTM had fewer injection-related adverse events than the clinician-selected SOC arm (standardized public figure: 4.8× fewer injection reactions).	Autologous origin generally limits allogeneic reaction risk. IFU lists contraindications: active infection, blood supply limitations, previous infections that may retard healing, hemodynamically unstable or hypercoagulable patients. As with any intra-articular injection, iatrogenic risks apply. (Dechra ProVet APC IFU)
Regulatory status (U.S.) How each is positioned	Working with FDA CVM to open the NADA; product currently manufactured under cGTP with transition to cGMP by end of Q3.	Cleared as a medical device for autologous point-of-care platelet-concentrate preparation. Not FDA-approved as a drug. IFU explicitly states safety and effectiveness for bone healing, hemostasis, or other clinical use “have not been established.” (Dechra IFU)
Consistency & QC What each dose looks like	Batch-tested finished product with a defined proteomic and EV profile from a closed-herd source (Coteau Grove Farms).	Cytokine and growth-factor concentrations characterized on a 6-horse cohort (Am J Vet Res 2024). Individual patient output will vary with donor platelet and WBC counts; the device produces the concentrate but does not standardize final composition across horses.
Practice economics Rep talking points	No capital equipment required. Single ready-to-use dose. Straightforward billing per treated horse.	Requires the portable ProVet centrifuge (~4.4 lbs) — designed for stall-side use — plus per-treatment disposables. Fast turnaround (<3 min centrifugation) is a workflow advantage over slower autologous platforms.

When to lead with equicenta® CTM	When ProVet® APC may be the better fit
• Case involves tendon or ligament alongside joint disease and the client wants a product with published outcomes across all three tissues. • Older, metabolic, or heavily pre-treated horse where autologous platelet quality is likely to be variable — a standardized allograft removes patient-side variability. • Practice wants peer-reviewed clinical efficacy evidence, not a characterization study, as the anchor for the treatment decision. • The vet wants 26-week durable performance data against a real-world active comparator, not a mechanistic rationale extrapolated from other PRP products. • The practice does not want to invest in an additional centrifuge platform or per-treatment disposables kit.	• Practice already owns the ProVet centrifuge and the case fits a fast stall-side autologous workflow. • Client or vet has an explicit preference for an autologous, blood-derived product rather than an allogeneic tissue biologic. • Case is a routine joint inflammation in a young, healthy horse where autologous platelet quality is expected to be strong. • Vet is choosing between L-PRP options and values the ProVet product characterization (high IL-1Ra:IL-1β ratio, elevated TGF-β1) as documented on a specific 6-horse cohort. • Same-visit turnaround (draw, spin, inject in one appointment) is the operational priority.

30-second positioning: “equicenta CTM and ProVet APC are often considered in the same conversation about joint biologics, but they’re fundamentally different tools. ProVet is a stall-side autologous platelet-concentrate device — a leukocyte-rich PRP where the composition depends on the individual horse’s blood on the day of treatment. equicenta CTM is an off-the-shelf allogeneic biologic with a consistent proteomic and EV profile from a closed herd, published clinical efficacy across joints, tendons, and ligaments in a 138-horse, 8-practice controlled trial, and a 26-week performance signal against real-world standard of care. If the practice already runs the ProVet workflow and the case is a routine joint in a healthy horse, ProVet is a reasonable choice. If tendon or ligament is involved, if the horse is older or heavily pre-treated, or if the practice wants clinical-efficacy evidence rather than a product-characterization study, lead with equicenta CTM.”

APPROVED WORDING RULES (ProVet APC conversations)

SAY	DON'T SAY
“ProVet APC is an autologous stall-side platelet-concentrate device; equicenta CTM is an allogeneic tissue biologic. Different tools for different cases.”	“ProVet doesn’t work” — ProVet’s output has been characterized in a peer-reviewed study (Am J Vet Res 2024) and is used widely in practice.

“The ProVet manufacturer’s IFU explicitly states that safety and effectiveness for bone healing, hemostasis, or other clinical use have not been established.”	“ProVet is unsafe” — the IFU disclosure is a regulatory clarity statement, not a safety concern.
“The ProVet peer-reviewed anchor is a characterization study on a 6-horse cohort; equicenta CTM is anchored in a 138-horse, 8-practice clinical-efficacy trial.”	“ProVet has no evidence” — characterize the evidence accurately (characterization vs. efficacy).
“Autologous L-PRP composition varies with the donor horse’s platelet and leukocyte counts; equicenta CTM removes patient-side variability with a batch-tested standardized product.”	“Autologous products are inconsistent” as a blanket claim — describe the specific driver (donor variability).
“In our multicenter controlled trial, eCTM outperformed clinician-selected standard of care on lameness, pain, swelling, and return to performance.”	“eCTM beat ProVet head-to-head” — the Bertone 2026 trial did not run a direct head-to-head vs. ProVet.
“equicenta CTM is off-the-shelf with no capital equipment required; ProVet is a portable stall-side workflow that runs on the ProVet centrifuge.”	Disparaging Dechra, the ProVet centrifuge, or the practice’s investment in it.

Sources. ProVet APC product literature and Instructions for Use: [Dechra ProVet APC IFU](#), [Dechra Canada product page](#). ProVet characterization: Am J Vet Res 2024 — [PubMed 41791222](#). equicenta CTM clinical evidence: Bertone AL, et al. 2026, Frontiers in Veterinary Science, DOI 10.3389/fvets.2026.1833933. Characterization & 42-day biocompatibility: BMC Medicine 2025. EV/exosome characterization: ACVSMR 2026 poster.

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